



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Vicarious Trauma

Reference #: FSFH -MENH-GU-01

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital Cockburn Clinic	Mental Health Service	Medical, Nursing, Allied Health, Administration, Security Services and Facilities management

1. Introduction

Vicarious trauma (VT) is a normal response to exposure to other people's trauma. Working to support people who have experienced trauma, and hearing, seeing, and learning about their experiences, can have an effect on staff and many aspects of their personal life. VT can affect all hospital staff including clinical, administrative and support staff¹. This document outlines the management of VT within the Mental Health Service (MHS) recognising that VT is an occupational hazard and a health and safety issue for all staff.

2. Terminology

Clinical Supervision	A key component of ethical and professional practice in the provision of mental health services where clinicians consult about their client work on a regular basis usually with a more experienced practitioner. It includes dimensions of support, encouragement of self-care and psychoeducation ² .
Reflective Practice	A process of thinking clearly, honestly, deeply, and critically about any aspect of our professional practice to improve the quality of care provided.
Trauma	A state of high arousal that impairs integration across many domains of learning and memory ² .
Vicarious Trauma (VT)	The psychological consequences of working with people who have experienced trauma. It is a state of physical, emotional

and mental exhaustion caused by long term involvement in emotionally demanding situations. It is a process through which health workers who are empathically engaged with patient/clients' trauma experience negative impacts on their emotional processing³

3. Policy

The MHS will:

- Promote the core principles of trauma-informed care which include 'safety', 'trustworthiness', 'choice', 'collaboration' and 'empowerment'² and how they are applied in practice.
- Provide education and training to staff working within the MHS on VT with particular emphasis on equipping managers to recognise, prevent and manage VT in themselves and their staff.
- Promote personal and professional strategies staff can use to reduce the risk of VT.
- Support the needs of staff managing VT.

4. Procedure

4.1. Recognising VT

- VT is similar to stress, trauma, compassion fatigue, burnout and secondary trauma stress however VT can actually change or impact a person's core beliefs and inner experiences.
- VT can lead to changes in a person's psychological and physical wellbeing and their outlook on the world. This may impact how they engage with:
 - Their own family and friends
 - The organisation
 - Patients and their families.
- The impact of VT is different for each person and each organisation.
- Staff who face stigma or discrimination based on their culture, ethnicity, gender or sexual orientation may be particularly vulnerable to VT.

Impact on staff	Impact on the organisation
Intrusive imagery, repeated and persistent flashbacks or nightmares, recollections of traumatic events that	Widespread cynicism and pessimism

will not stop.	
Negative changes to the way a staff member views the world, their spirituality and their identity.	Increased illness or absenteeism
Disruptions to personal and professional relationships resulting from feeling a loss of safety, trust, control, intimacy, and esteem.	Ethical or boundary violations
Difficulty managing feelings	Reduction in motivation, productivity, and commitment
Difficulties with decision making ⁴	High staff turnover
Physical symptoms may include fatigue, insomnia and nausea ¹	Decreased staff morale ²
Difficulty providing the best care to patients due to decreased compassion reserves, anxiety and depression ⁵ .	Patient outcomes and care compromised.

Table 1. Potential impacts of VT

4.2. Responding to VT

- Staff members who feel they may be experiencing VT are encouraged to:
 - Identify and implement those personal and professional strategies which may minimise the effects of VT.
 - Discuss concerns with their Line Manager and identify management strategies.
 - Seek peer support and debriefing.
 - Access the [Psychological Wellbeing and Support](#) (PWS) program
 - Access the [Employment Assistance Program](#) (EAP)
 - Complete a Combined Hospital or Incident Reporting (CHoIR) online.
- Line Managers and clinical supervisors who suspect a staff member is experiencing/ at risk of experiencing VT will:
 - Engage with the staff member in a confidential and empathetic manner.
 - Acknowledge that VT can affect anyone and outline areas of concern.
 - Discuss possible personal and professional strategies which may assist the staff member to manage.
 - Provide the staff member with PWS/EAP contact details.

- Establish and implement ongoing methods of supervision and support.
- Consider encouraging staff to complete a Professional Quality of Life Scale (PROQOL) to identify levels of VT, burnout and compassion fatigue.

Personal strategies
Awareness of the impact of VT on oneself and colleagues
Talk to someone about your feelings
Maintain a healthy balance between work and home life
Avoid working long hours, take regular leave
Set clear boundaries and limits
Allow yourself to slow down and take time out
Self-care and nurturing
Engaging in a healthy lifestyle e.g., exercise, healthy eating.
Maintain social interaction and connection.
Trying to maintain a broader perspective on current demands and issues.
Complete PROQOL scale on a regular basis to monitor for signs/symptoms (i.e. annually as part of PDR or in clinical supervision)
Professional strategies
Attending regular trauma informed clinical supervision or reflective practice sessions which includes review of VT.
Seeking out /using opportunities for debriefing with colleagues or supervisors (formal or informal)
Establish peer support networks through regular meetings
Developing a manageable balanced workload, ensuring breaks are taken
Maintaining professional development
Organisational strategies
New staff are provided with an orientation to their role and workplace.

Staff have equitable access to professional development opportunities both in house and external
Ensuring staff can take breaks
Ensuring fair and equitable distribution of workload
Encouraging supportive safe team environment
Time is set aside during staff meetings to discuss VT concerns
Supporting the prioritisation and attendance at discipline meetings
Supporting clinical supervision
Facilitating debriefing for staff and teams after responding to complex or distressing situations
A restorative just work culture open and supportive towards the issue of VT is promoted.
Provide training and education on VT to staff to increase awareness and encourage self-monitoring
Providing access to and encouraging use of EAP and PWS.

Table 2. Potential strategies for managing VT.

5. Compliance/performance monitoring

Compliance with this policy will be monitored by Line Managers and Discipline Leads through Performance Development Reviews, Clinical Supervision and Work Health and Safety reporting.

6. Related policy documents

SMHS:

- [Clinical supervision in mental health \(SMHS: 49\)](#)
- [Employee assistance program \(SMHS HR: 10\)](#)
- [Critical incident stress management](#)
- [Prevention of work-related stress](#)
- [Work Health and Safety SMHS: 123](#)

7. Related standards

- NSQHS
 - Clinical Governance

8. References

1. Saakvitne, K.W., & Pearlman, L. A., (1995) "Trauma and the Therapist: Countertransference and Vicarious Traumatization in Psychotherapy with Incest Survivors, WW Norton and Co, New York.
2. Kezelman, A. M. Stavropoulos, P. Practice Guidelines for Clinical Treatment of Complex Trauma (2019). National Centre of Excellence for Complex Trauma.
3. Pearlman, L.A., & McKay, L. (2009). Understanding and addressing vicarious trauma. On-line self-study module, Headington Institute, Pasadena, CA.
4. Turner, K., Stapelberg, N., Svetlicic, J. et al. (2020), Inconvenient truths in suicide prevention: Why a Restorative Just Culture should be implemented alongside a Zero Suicide Framework. Australian & New Zealand Journal of Psychiatry Vol. 54 (6) 571-581
5. Dodds H, Hunter DJ. Culture as both a risk and protective factor for vicarious traumatisation in nurses working with refugees: a literature review. Journal of Research in Nursing. 2022;27(4):357-371. doi:[10.1177/17449871221085863](https://doi.org/10.1177/17449871221085863)

9. Authorisation

EXECUTIVE SPONSOR: Nurse Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Authorised By	Revision due
1	04/2017	NUM, MH	Mental Health Clinical Practice Committee	Policy Committee	04/2020
2	04/2021	CNS, Clinical improvement	Service 5 SQR Committee	Service 5 SQR Committee	04/2024
3	07/2024	CNS, Clinical Improvement	Mental Health Clinical Practice Committee	Service 5 SQR Committee	07/2027